Reg. No. RMEE2552994
PURE PATH LAB
Add: Infront of Cantabil, Beside Distt. Hospital-Gonda
7703021000, 7897319900
www.purepathlab.in Gmail: help@purepathlab.in
Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779
PURE PATH LAB
Palce of Accuracy & Your Satisfaction

Mr. Jai Narayan
Age / Sex : 36 YRS / Male
Reg. no. : 7015

Registered on : -
Collected on : -
Received on : -
Reported on : -

SEROLOGY & IMMUNOLOGY
VIRAL MARKER

TEST	VALUE	UNIT	REFERENCE
HIV - 1	NEGATIVE	-	-
HIV - 2	NEGATIVE	-	-
HEPATITIS C VIRUS, HCV	NEGATIVE	-	-
HBSAG	NEGATIVE	-	-

HbsAg

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation

Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation

A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

SEROLOGY & IMMUNOLOGY
BIOCHEMISTRY

TEST	VALUE	UNIT	REFERENCE
C-REACTIVE PROTEIN, CRP (QUALITATIVE)	1.37	mg/L	< 6 mg/L
RANDOM BLOOD SUGAR	87.99	mg/dl	70 - 140

BIOCHEMISTRY
LIVER FUNCTION TEST (LFT)

TEST	VALUE	UNIT	REFERENCE
SERUM BILIRUBIN (TOTAL)	0.84	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)	0.16	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)	0.68	mg/dl	0.2 - 1
SGOT (AST)	35.39	U/l	0 - 37
SGPT (ALT)	38.15	U/l	13 - 40
SERUM ALKALINE PHOSPHATASE	134.8	U/l	44 - 147

BIOCHEMISTRY
LIVER FUNCTION TEST (LFT)

TEST	VALUE	UNIT	REFERENCE
SERUM PROTEIN	6.54	g/dl	6.4 - 8.3
SERUM ALBUMIN	3.87	g/dl	3.5 - 5.2
GLOBULIN	2.67	g/dl	1.8 - 3.6
A/G RATIO	1.45	1.1	- 2.1

LFT Interpretation

Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

Test Significance

Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.

Increased in

Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

BIOCHEMISTRY
KIDNEY FUNCTION TEST (KFT)

TEST		VALUE	UNIT	REFERENCE
SERUM UREA		40.56	mg/dl	19 - 45
SERUM CREATININE	**H**	**1.46**	**mg/dl**	**0.72 - 1.43**
SERUM PHOSPHORUS	**H**	**4.54**	**mg/dl**	**2.5 - 4.5**
SERUM URIC ACID	**H**	**7.24**	**mg/dl**	**3.5 - 7.2**
SERUM SODIUM		136.9	mmol/L	136 - 146
SERUM POTASSIUM		4.51	mmol/L	3.5 - 5.1
ICALCIUM		1.14	mmol/l	1.13 - 1.33

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia	1. Pregnancy
2. Large amount of dietary meat	2. Increasing age (reduction in muscle mass)
3. Active acromegaly and gigantism	

~~~ End of report ~~~
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